MEZO NETWORK: ASSESSOR'S FIELD GUIDE

Internal. For the person doing the visit, with the score sheet open on a tablet or printed.

THE RULES

1. **Seen, not said.** An item is met only on evidence you saw: a document, a log, a record, the thing itself working. "We do that" scores 0 until you see it.
2. **Score what is normal, not what is best.** A log kept for the last week but not the last three months scores 1.
3. **The assessor has no interest in the facility.** If you have one, stop and say so. A surgeon-owned facility is assessed by someone else.
4. **Write the evidence in the sheet as you go,** in a few words: "autoclave log Jul to Oct, BI weekly", "no capnograph in theatre 2".
5. **Photograph equipment and documents, never patients or identifiable records.**

PART A: WHAT COUNTS AS EVIDENCE

Ref	Met when you have seen
A1	The certificate, in date, naming the services in use
A2	The licence for the current year, and that this doctor is medical director of record here
A3	PCN and MLSCN certificates, or the signed arrangement with registered providers
A4	The NNRA licence, if X-ray or a C-arm is on site. Not applicable otherwise, scored Yes
A5	The policy schedule, and indemnity certificates for the theatre clinicians
A6	Completed checklists in all five traced records, signed at all three stages
A7	Pulse oximeter, BP and ECG at every theatre and recovery bed, working, and a capnograph wherever airways are used
A8	The rota and the anaesthetic charts of the traced cases show a named anaesthesia provider throughout
A9	The trolley, a working defibrillator (switch it on), and a daily check sheet signed for the last month
A10	Oxygen at each point, the reserve cylinders and their log, working suction
A11	The generator and changeover, and a battery backup for theatre monitors and lights. Ask when power last failed in theatre
A12	The autoclave, its cycle log, chemical indicators every load, biological indicator at least weekly
A13	The waste contract and colour-coded segregation in theatre and ward
A14	The written plan, and the transfer agreement with the named hospital and ambulance
A15	The blood arrangement in writing, and how long it actually takes
A16	Operative and anaesthetic notes and a consent form recording the risks in each traced record

Ref	Met when you have seen
A17	The named person, and where patient data is kept and who can see it
A18	The fire certificate in date, clear exits, extinguishers in date

PART B: THE FIVE-PATIENT TRACE

Choose five surgical patients from the last three months yourself from the theatre register, including the most recent major case and one that had a complication if there was one. For each, find:

- consent form with the risks discussed;
- pre-anaesthetic assessment;
- WHO checklist, all three stages;
- antibiotic given, and the time against the incision;
- anaesthetic chart and recovery chart;
- operative note;
- implant stickers, if an implant was used;
- the sterilisation load record for the instrument set;
- observations and early warning scores on the ward;
- venous thromboembolism assessment;
- discharge summary, and whether it went to the referring doctor.

Each missing link is a finding against B3, B4, B6 or B5. Record how many of the five had each link.

PART C: QUESTIONS THAT FIND THE REAL ANSWER

- "Show me the last time a patient deteriorated after surgery. What happened, and who decided?" (B5, A14)
- "If a batch of implants were recalled tomorrow, how would you find the patients?" (B6, Tier 3)
- "When did the power last go off during an operation?" (A11)
- "Who checks the trolley, and what happens if they are off?" (A9, B7)
- "How do you know your infection rate?" (B4, Tier 3)
- "Who covers the ward at 2am on a Sunday?" (B5, B7, Tier 2)

CLOSING THE VISIT

Tell the medical director what is missing and that they have five working days to send it. Do not give a Tier on the day. Score within two working days, have a second reviewer check any minimum marked No and any Tier 3 decision, and send the report within ten working days.